

IBS — Red Flags vs Supportive Features



1. IBS city gate / relapsing line

WHAT YOU SEE

Walled city labeled 'IBS' with a line of travelers entering and leaving.

WHAT IT ENCODES

IBS runs a relapsing-remitting course — symptoms come and go over years. A waxing/waning pattern SUPPORTS functional disease rather than a progressive organic process. Long, fluctuating history is reassuring.

2. 'Comes and goes' suitcase (gold star)

WHAT YOU SEE

A traveler with a gold star carrying a suitcase reading 'COMES AND GOES'.

WHAT IT ENCODES

Relapsing-remitting symptoms are a positive supportive feature of IBS. Gold star = correct/reassuring. Contrast with steadily progressive symptoms, which raise concern for organic disease.

3. Small volume / no blood (gold star)

WHAT YOU SEE

Traveler with gold star holding a sign 'SMALL VOLUME NO BLOOD'.

WHAT IT ENCODES

IBS-D produces SMALL-volume diarrhea WITHOUT blood. Large-volume or bloody/nocturnal diarrhea is NOT IBS. Absence of blood and small stool volume are supportive features.

4. Honest gate guard 75%/year

WHAT YOU SEE

Angry intestine gatekeeper next to a sign 'HONEST 75% SWITCH/YEAR'.

WHAT IT ENCODES

About 75% of IBS patients switch subtype (IBS-D ↔ IBS-C ↔ IBS-M) within a single year — so subtype switching is COMMON, not rare. Tested as a distractor against the 'switching is rare' claim.

5. Weight loss (red X)

WHAT YOU SEE

Gaunt man with a tag 'WEIGHT LOSS' marked with a red X.

WHAT IT ENCODES

Unintentional weight loss is a RED FLAG arguing against IBS — mandates workup for malignancy, IBD, or malabsorption. Functional IBS does not cause weight loss.

BOARD TRAP

"Weight loss is consistent with IBS" → wrong; it is an alarm feature mandating investigation.

6. Nocturnal pain (red X)

WHAT YOU SEE

Man in striped pajamas under a moon, clutching abdomen, red X over chest.

WHAT IT ENCODES

Pain that wakes the patient at night is an organic red flag — functional IBS pain typically does not disrupt sleep. Nocturnal symptoms mandate evaluation (colonoscopy, calprotectin, celiac serology).

BOARD TRAP

"Nocturnal pain fits IBS" → wrong; night-waking pain is a red flag for organic disease.

7. Nocturnal diarrhea (red X)

WHAT YOU SEE

Man holding an alarm clock, red X, running with stool symbol.

WHAT IT ENCODES

Diarrhea that wakes the patient from sleep (nocturnal diarrhea) is a red flag pointing to organic disease such as IBD or infection. IBS-D does not characteristically occur at night.

BOARD TRAP

"Nocturnal diarrhea is typical of IBS" → wrong; it is an alarm feature requiring workup.

8. New onset in older age (red X)

WHAT YOU SEE

Elderly man with a cane and sign 'NEW IN OLD AGE', red X.

WHAT IT ENCODES

New-onset symptoms in an older patient (e.g., >50) are a red flag — colorectal cancer and other organic disease rise with age, mandating colonoscopy. IBS usually presents in younger adults.

BOARD TRAP

"New IBS commonly starts in the elderly" → wrong; new onset in older age mandates workup for organic disease.

9. Steatorrhea (red X)

WHAT YOU SEE

Far-right traveler holding a greasy stool symbol, red X.

WHAT IT ENCODES

Steatorrhea (fatty, floating, foul stool) signals fat malabsorption — celiac, pancreatic insufficiency, or small-bowel disease — not IBS. A red flag mandating malabsorption workup.

BOARD TRAP

"Steatorrhea is part of IBS-D" → wrong; it indicates malabsorption and needs investigation.

10. 'Switching is rare' agitator (red X)

WHAT YOU SEE

Man holding a placard 'SWITCHING IS RARE' looking confused/rejected.

WHAT IT ENCODES

FALSE claim: subtype switching is rare. In reality ~75% of patients switch subtype within a year, so switching is COMMON. This is the explicit board trap on this scene.

BOARD TRAP

"Switching between diarrhea and constipation is rare" → wrong; ~75% of patients switch subtype within a year.
